

STATISTICAL ANALYSIS PLAN

A Phase 2, Open-Label, Randomized, Non-Comparative Study with Preliminary Dose-Finding to Evaluate eFT508 Monotherapy or eFT508 in Combination With Avelumab in Subjects With Microsatellite Stable Relapsed or Refractory Colorectal Cancer

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Protocol Version: 4.0
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List of Abbreviations and Definitions of Terms

ABBREVIATION	DEFINITION
AE	adverse event
aPTT	Activated partial thromboplastin time
ATC	Anatomical therapeutic chemical
BID	<i>Bis in die</i>
BMI	Body mass index
BOR/iBOR	Best objective response
CI	Confidence interval
CR/iCR	Complete response
CT	Computed tomography
CTCAE	Common Terminology Criteria for Adverse Events
DCR	Disease control rate
DLT	Dose limiting toxicity
DR/iDR	Duration of response
DSD/iDSD	Duration of stable disease
ECOG	Eastern Cooperative Oncology Group
ECG	Electrocardiogram
eCRF	Electronic Case Report Form
iCPD	Confirmed disease progression assigned using iRECIST
iUPD	Unconfirmed disease progression assigned using iRECIST
iRECIST	Modified Response Evaluation Criteria in Solid Tumor in cancer immunotherapy trials
MedDRA	Medical Dictionary for Regulatory Activities
MRI	Magnetic resonance imaging
MTD	Maximum tolerable dose
NE	Not evaluable
ORR/iORR	Objective response rate
OS	Overall survival
PBMC	Peripheral blood mononuclear cells
PD	Progressive Disease
PET	Positron emission tomography
PFS/iPFS	Progression free survival
PK	Pharmacokinetics
PO	<i>Per os</i>
PR/iPR	Partial response
PT	Preferred Term
RECIST	Response Evaluation Criteria in Solid Tumors
SAE	Serious adverse event
SAP	Statistical Analysis Plan
SD/iSD	Stable disease
SOC	System Organ Class

ABBREVIATION	DEFINITION
TEAE	Treatment-emergent adverse event
TESAE	Treatment-emergent serious adverse event
WHO	World Health Organization

1 INTRODUCTION

This Statistical Analysis Plan (SAP) describes the statistical methodology and analyses to be conducted for eFFECTOR Therapeutics, Inc. protocol eFT508-0006 (Version 4.0; 17 December 2018). Any deviations from this analysis plan will be substantiated by sound statistical rationale and will be documented in the final clinical study report.

2 STUDY OBJECTIVES

2.1 Part 1

Primary Objective:

- To determine the MTD and select a recommended dose of eFT508 in combination with avelumab for Part 2 in subjects with microsatellite stable relapsed or refractory colorectal cancer.

Secondary Objectives:

- To characterize the safety and tolerability profile of eFT508 in combination with avelumab in this study population.
- To evaluate the PK of eFT508 and avelumab when used in combination in this study population.
- To assess the antitumor activity of eFT508 in combination with avelumab in this study population using Response Evaluation Criteria in Solid Tumors (RECIST) 1.1 and iRECIST.
- To evaluate the immunogenicity of the eFT508 in combination with avelumab.

Exploratory Objectives:

- To assess target engagement of eFT508 in peripheral blood mononuclear cells (PMBCs) and tumor tissue.
- To assess the mechanism of action of eFT508.
- To assess predictive markers for sensitivity and resistance to eFT508.

2.2 Part 2

Primary Objective:

- To assess the antitumor activity of eFT508 in combination with avelumab in subjects with microsatellite stable relapsed or refractory colorectal cancer using RECIST 1.1 and iRECIST.
- To assess the antitumor activity of eFT508 monotherapy in subjects with microsatellite stable relapsed or refractory colorectal cancer using RECIST 1.1 and iRECIST.

Secondary Objectives:

- To assess safety and tolerability of eFT508 monotherapy and combination therapy with avelumab in this study population.
- To further characterize the PK of eFT508 and avelumab when used in combination in this study population.
- To evaluate the immunogenicity of eFT508 in combination with avelumab.

Exploratory Objectives:

- To assess target engagement of eFT508 in PBMC and tumor tissue.
- To assess the mechanism of action of eFT508.
- To assess predictive markers for sensitivity and resistance to eFT508.
- To assess the antitumor activity of eFT508 in combination with avelumab in subjects who fail to achieve an objective response to eFT508 monotherapy using RECIST 1.1 and iRECIST.

3 STUDY OVERVIEW

3.1 Study Design

This Phase 2, open-label, 2-part, multicenter study will evaluate the safety, efficacy, PK, and pharmacodynamics of eFT508 in combination with avelumab in subjects with microsatellite stable relapsed or refractory colorectal cancer.

The primary objective of Part 1 is to evaluate the safety and tolerability of escalating doses of eFT508 in combination with a fixed dose of avelumab to determine the MTD of eFT508 and to select a recommended dose of eFT508 in combination with avelumab for Part 2. The primary objective of Part 2 is to evaluate the efficacy of eFT508 at the recommended dose in combination with avelumab or eFT508 monotherapy. Parts 1 and 2 will also evaluate PK and pharmacodynamics.

Study therapy will be administered in 4-week treatment cycles. eFT508 will be taken orally (PO) bid. Avelumab 10 mg/kg will be administered IV on Day 1 and q2wk thereafter. Study therapy will be administered until documented disease progression by iRECIST, unacceptable toxicity, the subject withdraws consent, the Investigator or Sponsor discontinues the subject, or the study is terminated. Subjects who discontinue study therapy permanently will remain on study for the acquisition of safety information for up to 30 days after the last dose of study therapy. Thereafter, subjects will be followed at 3- to 6-month intervals, at the Sponsor's discretion, for up to 3 years to collect information on the initiation of first subsequent anticancer therapy and survival.

Safety assessments will include adverse event (AE) recording, physical examinations, vital sign measurements, ECGs, Eastern Cooperative Oncology Group (ECOG) performance status, and clinical safety laboratory tests (hematology, serum chemistry, thyroid function, coagulation, and urinalysis).

Efficacy assessments will include radiographic examinations (and physical examinations if needed) every 8 weeks. Radiology examinations include CT (and may include positron emission tomography [PET], if relevant) or magnetic resonance imaging (MRI) imaging of all sites of measurable disease. For subjects with bone metastases, a bone scan (MRI allowed as alternative) should be performed every 16 weeks. Disease response will be evaluated using RECIST 1.1 and iRECIST.

This study will also analyze eFT508 and avelumab PK and pharmacodynamics. Pharmacodynamic analyses will explore target engagement of eFT508, the mechanism of action of eFT508, and predictive markers for sensitivity and resistance to eFT508.

Note: All subjects will undergo a fresh pre-treatment tumor biopsy and a post-treatment biopsy for pharmacodynamic analysis. Genetic analysis may also be performed on blood and biopsy tissue samples. Genetic testing is optional. Genetic analysis will include T-cell clonality and the analysis of relevant oncogenic mutations including, but not limited to, KRAS and p53.

3.1.1 Dose Escalation in Part 1

In Part 1, a Dose Limiting Toxicity (DLT) is defined as an AE meeting specific criteria and not primarily related to the underlying malignancy, a comorbid condition, or an intercurrent illness. The full list of criteria for determination of DLTs is given in Protocol Section 3.2.3.

3.1.1.1 Dose-Escalation Guidelines

A standard 3+3 dose-escalation design will be used in Part 1. Dose escalation will be guided by the safety data from the first treatment cycle (Day 1 through Day 28) in each cohort.

In each cohort, 3 subjects will be treated with eFT508 and avelumab.

If 0 of 3 subjects experiences a DLT during Cycle 1, then enrollment in the next higher dose cohort will begin.

If 1 of 3 subjects experiences a DLT, then 3 additional subjects will be treated in that dose cohort. If 0 of the next 3 subjects experiences a DLT, enrollment in the next higher dose cohort will begin.

If ≥ 2 of 3 or ≥ 2 of 6 subjects experience a DLT, that cohort will be declared intolerable, and no further dose escalation will occur. If this occurs in the first cohort, an alternative qd schedule will be considered.

The following dose escalation progression will be utilized in Part 1.

Table 1: Dose Cohorts in Part 1

Cohort	eFT508		Avelumab	Number of Subjects
	Dose bid (mg)	Dose Change Relative to Prior Dose	Dose (mg/kg)	
1 (Starting Dose Level)	100	NA	10	3-6
2	200	2.0	10	3-6
3	300	1.5	10	3-6
4	400	1.3	10	3-6

Abbreviations: bid, twice a day; NA, not applicable.

Note: eFT508 will be taken bid at 12-hour intervals (± 2 hours).

An End-of-Cohort safety review will be conducted by the Investigators and Medical Monitor after the last subject has completed the first treatment cycle in each cohort to determine if dose escalation may proceed.

3.1.1.2 Maximum Tolerable Dose

In Part 1, the maximum tolerable dose (MTD) will be the highest eFT508 dose level evaluated at which 1 or fewer of 6 subjects experienced a DLT in Cycle 1.

The recommended dose for Part 2 may be the MTD or may be a lower dose. Selection of the recommended dose will be based on consideration of short- and long-term safety information together with the available PK, pharmacodynamics, and efficacy data.

3.2 Study Centers

This study will be conducted at 6 to 10 study sites in the United States.

3.3 Randomization and Blinding

In Part 2, eligible subjects will be randomized in a ratio of approximately 2:1 to receive either eFT508 in combination with avelumab or eFT508 monotherapy. Randomization will be implemented centrally through an interactive voice response/interactive web response. Subjects will be directly assigned a treatment arm as this is an open-label study.

This is an open-label study; therefore the Sponsor, Investigator, and subject will know the study therapy being administered.

3.4 Sample Size Determination

In Part 1, up to approximately 30 DLT-evaluable subjects will be enrolled. The total number of subjects will depend upon the number of dose cohorts evaluated and the number of subjects treated in each cohort.

In Part 2, up to approximately 40 efficacy evaluable subjects will be enrolled and randomized in approximately a 2:1 ratio to receive eFT508 in combination with avelumab (27 subjects) or eFT508 monotherapy (13 subjects). In the combination arm, the Simon 2-stage mini-max design to test the null hypothesis that the ORR is $\leq 5\%$ against the one-sided alternative hypothesis that it is $\geq 20\%$ will be used.

Under this design, 13 subjects will be accrued in the first stage. If 0 of 13 subjects have an objective tumor response per iRECIST, then randomization to both treatment arms will cease and the study will stop. If one or more objective tumor responses per iRECIST are seen in Stage 1, 14 additional subjects would be accrued in Stage 2 for a total of 27 subjects. The null hypothesis will be rejected if ≥ 4 objective responses are observed in the total of 27 subjects (15% ORR) by iRECIST. This design yields a type I error rate of 0.0416 and a power of 0.8011 when the true ORR is 20%.

3.5 Duration of Study

The overall duration of the study is anticipated to be approximately 2 years from the first subject's first visit until the last subject's last visit. For regulatory reporting purposes, the end of the study is defined as the date the last subject completes their End of Therapy Visit or the last subject completes 6 months of therapy, whichever occurs first.

3.6 Schedule of Events

The Schedule of Events is presented in Table 2.

Table 2: Schedule of Events

[illegible]

Abbreviations: ACTH, adrenocorticotrophic hormone; AE, adverse event; d, day; D, day; ECG, electrocardiogram; ECOG, Eastern Cooperative Oncology Group; iCPD, confirmed progression assigned using iRECIST; iUPD, unconfirmed progression assigned using iRECIST; TTNT, time to next therapy; WOCBP, women of childbearing potential.

[a] **Physical exam:** Height will also be measured at this visit.

[b] **Weight:** Subjects on combination therapy should be weighed on the day of each avelumab dosing, prior to the infusion. If the subject experiences either a weight loss or gain >10% compared to the weight used for the last dose calculation, the amount of avelumab must be recalculated using the most recent weight obtained.

[c] **Vital signs:** Vital signs should be taken with the subject resting in a supine position and before study therapy is administered.

[d] **ECG:** ECGs should be performed with the subject resting in a supine position.

[e] **AE assessment:** AEs should be recorded from the date of first dosing until 30 days after the last dose of study therapy. After discontinuing study therapy, subjects will be followed until the later of either 30 days after the last dose of study therapy or until resolution/stabilization of any ongoing drug-related AEs. This follow-up may be obtained in person or by telephone contact.

[f] **Concomitant medications:** Only medications to treat AEs will be recorded for the Post-Therapy Follow-Up.

[g] **eFT508 dosing:** Subjects will take their morning dose of eFT508 at the site at each study visit. The time of the dose must be recorded. All other eFT508 doses will be taken at home. Subjects will also record the time they take their last eFT508 dose the day before each study visit on the Instructions to Patient Sheet through Cycle 8. For subjects on combination therapy, the morning dose of eFT508 should be taken 30 minutes (± 5 minutes) before avelumab premedication on avelumab infusion days. If premedication is no longer required, eFT508 will be administered 30 to 60 minutes before the avelumab infusion.

[h] **eFT508 compliance:** Empty, partially used, or full bottles of eFT508 will be retrieved from the subject. Sites will review the Instructions to Patient Sheet for eFT508 compliance.

[i] **Avelumab infusion (subjects on combination therapy):** In order to mitigate infusion-related reactions, premedication should be administered 30 to 60 minutes prior to each avelumab infusion. The avelumab infusion will be 50 to 80 minutes. The start and stop time of the infusion should be recorded. Subjects should remain at the site for at least 2 hours after each infusion of avelumab. If, following the 4th dose of avelumab, no infusion-related reactions are observed, no further premedication will be required for subsequent administrations and subjects will require only 1 hour of observation following the infusion.

[j] **Pregnancy testing:** Serum pregnancy testing is required during screening; serum or urine pregnancy testing is permitted thereafter. Investigators must confirm that the pregnancy test results are negative before study therapy is administered.

[k] **Sample collection:** During the Treatment Period, blood and urine samples should be taken ≤ 48 hours before study therapy is administered.

[l] **Hematology results:** For subjects on combination therapy, Investigators must confirm that the hematology results are acceptable ($ANC \geq 1.0 \times 10^9/L$ and platelet count $\geq 50 \times 10^9/L$) before initiating the avelumab infusion.

[m] **Thyroid function and ACTH tests:** Cycle 3 and every 8 weeks thereafter (ie, Cycles 5, 7, 9, etc.)

[n] **eFT508 PK:** Blood samples will be collected before the eFT508 morning dose and at 3 ± 1 hours after the eFT508 morning dose.

[o] **eFT508 PK, eFT508 pharmacodynamics and optional genetic testing:** A blood sample will be collected at 6 ± 2 hours after the eFT508 morning dose to coincide with biopsy collection time.

[p] **eFT508 and avelumab PK and eFT508 pharmacodynamics:** Cycles 4 and 8 only.

[q] **Avelumab PK (subjects on combination therapy) and pharmacodynamics (all subjects):** Blood samples will be collected before the eFT508 morning dose.

[r] **Optional genetic testing (blood):** Subjects will be asked to consent to provide blood samples for optional genetic testing. Genetic testing will include T-cell clonality and the analysis of relevant oncogenic mutations including, but not limited to, KRAS and p53. [s] **Anti-avelumab antibodies (subjects on combination therapy):** Blood samples will be collected before eFT508 dosing. Samples that are positive for anti-avelumab antibodies will undergo characterization for neutralizing antibodies. [t] **Fresh tissue biopsy:** All subjects will undergo a fresh pre-treatment tissue biopsy for pharmacodynamic assessments (by contract laboratories). Note: existing material may be used to determine study eligibility. A post-treatment biopsy will be taken 6 ±2 hours after the eFT508 morning dose on Cycle 2, Day 1 for pharmacodynamic analysis. The post-treatment biopsy should be taken from the same lesion as the pretreatment biopsy (preferred) or from the same organ. See Study Laboratory Manual for tissue sample requirements. Subjects will be asked to consent to optional genetic analysis of biopsy tissue.

[u] **Disease assessment on study:** The preferred radiology imaging methods are CT, CT/PET, or MRI of all sites of measurable disease. During the Treatment Period, radiology imaging should be performed ≤7 days before initiating Cycles 3, 5, 7, 9, etc. (ie, 8-week intervals). For subjects with bone metastases, a bone scan (MRI allowed as alternative) should be performed every 16 weeks. Subjects who do not have iCPD, as defined by iRECIST, will be eligible to continue study therapy. Any partial or complete response must be confirmed ≥4 weeks after the response is first documented. If iUPD is observed, a confirmatory scan should be done within 4-8 weeks per iRECIST.

[v] **Disease assessment at the End of Therapy Visit:** This assessment should be performed unless the subject had iCPD documented. See Study Protocol Appendix 13.6 and Appendix 13.7 for RECIST and iRECIST guidelines.

[w] **Long-term follow-up:** Information about survival status and the first subsequent anticancer therapy will be collected at approximately 3- to 6-month intervals at the Sponsor's discretion through 3 years. This information will be gathered during routine clinic visits, other study site contact with the subjects, or via telephone or e-mail with the subjects/caregivers or referring physician offices.

4 ANALYSIS POPULATIONS

This section defines the analysis populations to be used for the planned statistical analyses.

4.1 Enrolled Population

All enrolled subjects in Part 1 and Part 2 will be included in the Enrolled Population.

In Part 2, randomized subjects will be considered enrolled.

The Enrolled Population will be used for select study population summaries and disposition summaries.

4.2 Safety Population

The Safety Population in Part 1 and Part 2 includes all subjects who receive at least one dose of study therapy (i.e. at least one dose of eFT508 or avelumab).

The Safety Population will be used for all demographics and baseline characteristics, safety and efficacy analyses, as well as all analyses of pharmacodynamic endpoints.

4.3 DLT Evaluable Population

The Dose Limiting Toxicity (DLT) Evaluable Population includes all subjects in Part 1 who meet the definition of DLT Evaluable.

Subjects will not be considered evaluable for DLT if they meet any of the following criteria:

- Had a major deviation including failure to satisfy major entry criteria (eg, confirmation of disease, signed informed consent).
- Received <44 of 56 eFT508 doses or less than 2 doses of avelumab in Cycle 1 for reasons other than the occurrence of a DLT or drug intolerance.
- Received other antitumor treatments during Cycle 1.

The DLT Evaluable Population will be used in the determination of the MTD in Part 1.

5 STUDY ASSESSMENTS

5.1 Efficacy Assessments

5.1.1 Clinical Response Assessment (RECIST)

The treatment response for each subject will be determined at the timepoints in the Schedule of Events using RECIST v1.1 guidelines. At each assessment, subjects will be assigned a treatment response by the Investigator from among the following RECIST categories:

- Complete Response (CR)
- Partial Response (PR)
- Stable Disease (SD)
- Progressive Disease (PD)
- Not Evaluable (NE)

Definitions for the above categories are described in Protocol Section 13.6.4.

5.1.2 Clinical Response Assessment (iRECIST)

The treatment response for each subject will also be determined at the timepoints in the Schedule of Events using iRECIST guidelines. At each assessment, subjects will be assigned a treatment response by the Investigator from among the following iRECIST categories:

- Complete Response (iCR)
- Partial Response (iPR)
- Stable Disease (iSD)
- Unconfirmed Progressive Disease (iUPD)
- Confirmed Progressive Disease (iCPD)
- Not Evaluable (NE)

In addition, if the iRECIST assessment is considered iCPD, then the confirmed progressive disease will be classified according to one of the following subcategories

- Further increase in sum of target lesions $\geq 5\text{mm}$
- Further increase in sum of new target lesions $\geq 5\text{mm}$
- Another new lesion
- Significant further increase in non-target lesions

Further details regarding iRECIST response assessments are given in Protocol Section 13.7.

5.1.3 Pharmacodynamic Assessments

Blood samples for pharmacodynamics (PD) will be obtained at the times indicated in the Schedule of Events.

All enrolled subjects will have a pre- and post-treatment biopsy collected. Note: pre-existing biopsy tissue may be used to determine study eligibility. In the event a pre-existing biopsy tissue is not available, microsatellite instability (MSI) expression will be analyzed on the pre-treatment biopsy tissue by immunohistochemistry (IHC) or polymerase chain reaction (PCR)-based tests and the results must be available prior to enrollment.

As in the Schedule of Events the tumor biopsy will be taken during the Screening Period and on Cycle 2 Day 1.

5.2 Safety Assessments

Safety assessments will include adverse event reporting, clinical laboratory evaluations (including chemistry and hematology), vital signs, Electrocardiograms (ECGs), physical examinations, ECOG performance status evaluation, and prior and concomitant medication reporting.

5.2.1 Adverse Events

An adverse event (AE) is defined as any untoward medical occurrence in a clinical investigation subject administered a study drug; the event does not necessarily have a causal relationship with study drug or usage.

All AEs regardless of causality or seriousness will be recorded from the date of first dosing until the later of either 30 days after the last dose of study therapy or until resolution/stabilization of any ongoing drug-related AEs.

AEs that start during or after initiating study therapy, or AEs with an onset prior to initiating study therapy that worsen after study therapy initiation will be considered treatment-emergent adverse events (TEAEs). Initiation of study therapy will be the first dose date of eFT508 and/or avelumab. AEs that occur between the time the subject signs the informed consent form and the start of study therapy will be considered pre-treatment AEs.

AEs will be coded using the Medical Dictionary for Regulatory Activities (MedDRA Version 21.0). The severity of all adverse events will be graded according to NCI CTCAE version 4.03. If an AE is not given a severity grade it will not be imputed.

The relationship of an AE to the administration of eFT508 and/or avelumab will be classified as possibly related or unlikely related as follows.

Table 3: Relationship of an Adverse Event to Study Drug

Relationship	Description
Possibly Related	The AE is known to occur with the study drug, there is a reasonable possibility that the study drug caused the AE, or there is a temporal relationship between the study drug and event. Reasonable possibility means that there is evidence to suggest a causal relationship between the study drug and the AE.
Unlikely Related	There is not a reasonable possibility that the administration of the study drug caused the event, there is no temporal relationship between the study drug and event onset, or an alternate etiology has been established.

5.2.2 Clinical Laboratory Evaluations

Clinical laboratory assessments will be performed at the visits and times specified in the Schedule of Events (Table 2).

The following clinical laboratory parameters will be evaluated during the study.

Table 4: Diagnostic and Safety Laboratory Assessments and Parameters

Laboratory Test	Parameters
Pregnancy Test	• β -HCG (serum test required during screening; serum or urine tests permitted thereafter)
Virology	• Serum HIV antibody • Serum HbsAg antibody, HBc antibody (if positive, then serum HBV DNA by PCR) • Serum HCV antibody (if positive, then serum HCV RNA by PCR)
Serum Chemistry	• Sodium, potassium, chloride, bicarbonate, BUN, creatinine, uric acid, glucose, calcium, phosphorus, magnesium, total protein, albumin, ALT, AST, ALP, CK, LDH, total bilirubin
Hematology	• Hematocrit, hemoglobin, erythrocyte count

	• Absolute counts of leukocytes, neutrophils, lymphocytes, monocytes, eosinophils, basophils • Platelet count
Coagulation	• Prothrombin time, Activated partial thromboplastin time (aPTT)
Thyroid function and ACTH	• TSH, T3, Free T4, and ACTH tests
Urinalysis	• Dipstick: specific gravity, pH, protein, glucose, ketones, bilirubin, urobilinogen, blood, nitrite, leukocyte esterase • Microscopy: white blood cells, red blood cells, epithelial cells, bacteria, casts, crystals

5.2.3 Weight and Height

Weight (kg or lb) and Height (cm or inches) will be assessed at Screening.

Weight will also be assessed after Screening at the times indicated in the Schedule of Events.

5.2.4 Vital Signs

Vital signs will be assessed at the times indicated in the Schedule of Events.

Vital signs measurements including systolic and diastolic blood pressure (mmHg), pulse rate (beats/min), respiratory rate (breaths/min), and temperature (°C or °F).

5.2.5 Electrocardiograms (ECGs)

A 12-lead Electrocardiogram will be completed at the times indicated in the Schedule of Events. The ventricular rate (beats/min), QRS interval (msec), QT interval (msec), QTc interval (msec), PR interval (msec), RR interval (msec), and overall interpretation (Normal, abnormal clinically significant, abnormal not clinically significant) will be recorded.

5.2.6 Physical Examinations

Physical Examinations will be performed at the times indicated in the Schedule of Events. Any abnormalities will be noted as AEs and corresponding information will be recorded in the eCRF.

5.2.7 Eastern Cooperative Oncology Group

Eastern Cooperative Oncology Group (ECOG) performance status will be assessed at the times indicated in the Schedule of Events. The ECOG performance status on a scale from 0-4 will be recorded.

5.2.8 Prior and Concomitant Medications

All medications taken by subjects between signing the informed consent form and the End of Therapy Visit will be recorded. Thereafter, only concomitant medications taken for the treatment of an AE will be recorded for up to 30 days after the last dose of study therapy. Concomitant medications include all prescription, over-the-counter, IV infusions, transfusions, and alternative medications (including herbal products and vitamins).

All medications will be recorded on the electronic case report form (eCRF) and will be coded using the most current version of the World Health Organization Drug Dictionary (WHO-DDE B2, March 2017 or later).

Prior medications are defined as medications that were started prior to the first dose date of study drug (eFT508 and/or avelumab). Concomitant medications are medications that were taken on or after the first dose date of study drug. A medication can be both prior and concomitant if it was started prior to the first dose date of study drug and continued during treatment with eFT508 or avelumab.

The medication administered, indication, dose, route, frequency, and start and stop dates will be recorded in the eCRF.

5.2.9 Immunogenicity

Blood samples will be collected to evaluate anti-avelumab antibody development.

5.3 Pharmacokinetic Assessments

Blood samples for Pharmacokinetic (PK) assessments of eFT508 and avelumab will be obtained at the times indicated in the Schedule of Events.

6 CLINICAL OUTCOME VARIABLES

This section describes the clinical outcomes and derivation of variables considered for this study.

6.1 Primary Efficacy Endpoint

6.1.1 Objective Response Rate

In Part 2, the primary efficacy endpoint will be the Objective Response Rate (ORR/iORR) using RECIST v1.1 and iRECIST based on the Safety Population.

BOR will be determined based on the overall visit responses from each RECIST/iRECIST assessment by the Investigator. BOR/iBOR is defined as the best response (in the order of CR, PR, SD, PD for RECIST and iCR, iPR, iSD, iUPD, and iCPD using iRECIST) a subject has experienced following the first dose of study treatment, but prior to and including progression or the last evaluable assessment in the absence of disease progression. PR/CR (or iPR/iCR using iRECIST) must be confirmed. Subjects with no evaluable RECIST/iRECIST assessments after first dose of eFT508 will be assigned to the NE category, unless the reason for discontinuation is clinical progression. If a subject has no evaluable RECIST/iRECIST assessments after first dose of eFT508 and discontinues due to clinical progression, the BOR will be assigned to the PD/iUPD category.

Using RECIST v1.1, the ORR is defined as the proportion of subjects whose BOR is a CR or PR. The CR or PR must be confirmed at least 4 weeks after the response is first documented to be included.

Using iRECIST, the iORR is defined as the proportion of subjects whose iBOR is an iCR or iPR. The iCR or iPR must be confirmed at least 4 weeks after the response is first documented to be included.

The determination of the best overall response by RECIST v1.1 is summarized in the table below:

Table 5: Best Overall Response RECIST Assessment Considering Requirement for Confirmation

Response Category at First Timepoint	Response Category at Subsequent Timepoint	Best Overall Response
CR	CR	CR
CR	PR	PD unless minimum criteria for SD duration met ^[a] , in which case SD (possible PR ^[b])
CR	SD	SD if minimum criteria for SD duration met ^[a] , otherwise PD if no further assessment
CR	PD	SD if minimum criteria for SD duration met ^[a] , otherwise PD if no further assessment
CR	NE	SD if minimum criteria for SD duration met ^[a] , otherwise NE
PR	CR	PR
PR	PR	PR
PR	SD	SD
PR	PD	SD if minimum criteria for SD duration met ^[a] , otherwise PD if no further assessment
PR	NE	SD if minimum criteria for SD duration met ^[a] , otherwise NE
NE	NE	NE
SD	CR	SD
SD	PR	SD
SD	SD	SD
SD	PD	SD if minimum criteria for SD duration met ^[a] , otherwise PD if no further assessment
SD	NE	SD if minimum criteria for SD duration met ^[a] , otherwise NE
PD	CR, PR, SD	SD if minimum criteria for SD duration met ^[a, c] , otherwise PD if no further assessment
PD	PD	PD
PD	NE	PD if no further assessment
NE	CR	SD
NE	PR	SD
NE	SD	SD
NE	PD	PD if no further assessment
NE	NE	NE

Abbreviations: BOR, best overall response; CR, complete response; NE, nonevaluable; PD, progressive disease; PR, partial response; SD, stable disease.

^[a] Assignment of SD requires that ≥ 1 post-baseline scan was obtained ≥ 4 weeks from start of study therapy and met criteria for SD or better response.

^[b] If a true CR occurs at the first timepoint, then any disease seen at a subsequent timepoint (even disease meeting PR criteria relative to baseline) results in a BOR of PD (because the disease must have reappeared after the CR). The BOR assessment would depend on whether the minimum duration for SD was met. For a subject who retrospectively had only an apparent CR (with small lesions still present, ie, the subject had PR, not CR, at the first timepoint, the original CR should be converted to a PR and the BOR should be assessed as PR

^[c] Transient worsening of disease early in therapy or during temporary interruption of study therapy (eg, for drug-related toxicity or intercurrent illness) may not indicate true PD if followed by an SD, PR, or CR.

Table 6: Best Overall IRECIST Response Assessment Considering Requirement for Confirmation

Response Category at First Timepoint	Response Category at Subsequent Timepoint	Best Overall Response
iCR	iCR	iCR
iCR	iPR	iUPD unless minimum criteria for iSD met ^[a] , in which case iSD (possible iPR ^[b])
iCR	iSD	iSD if minimum criteria for iSD met ^[a] , otherwise iUPD if no further assessment
iCR	iUPD	iSD if minimum criteria for iSD met ^[a] , otherwise iUPD if no further assessment
iCR	NE	iSD if minimum criteria for iSD met ^[a] , otherwise NE
iPR	iCR	iPR
iPR	iPR	iPR
iPR	iSD	iSD
iPR	iUPD	iSD if minimum criteria for iSD met ^[a] , otherwise iUPD if no further assessment
iPR	NE	iSD if minimum criteria for iSD met ^[a] , otherwise NE
NE	NE	NE
iSD	iCR	iSD
iSD	iPR	iSD
iSD	iSD	iSD
iSD	iUPD	iSD if minimum criteria for iSD met ^[a] , otherwise iUPD if no further assessment
iSD	NE	iSD if minimum criteria for iSD met ^[a] , otherwise NE
iUPD	iCR, iPR, iSD	iSD if minimum criteria for iSD met ^[a,c] , otherwise iUPD if no further assessment

iUPD	iCPD	iCPD
iUPD	NE	iUPD if no further assessment
NE	iCR	iSD
NE	iPR	iSD
NE	iSD	iSD
NE	iUPD	iUPD if no further assessment
NE	NE	NE

Abbreviations: iBOR, best overall response; iCR, complete response; NE, not evaluable; iUPD, unconfirmed progressive disease; iCPD, confirmed progressive disease; PR, partial response; SD, stable disease.

- [a] Assignment of iSD requires that at least one post-baseline scan was obtained at least 4 weeks from start of study therapy and met criteria for iSD or better response.
- [b] If a true iCR occurs at the first timepoint, then any disease seen at a subsequent timepoint (even disease meeting PR criteria relative to baseline) results in a iBOR of iUPD (because the disease must have reappeared after the iCR). The BOR assessment would depend on whether the minimum duration for iSD was met. For a subject who retrospectively had only an apparent iCR (with small lesions still present, i.e. the subject had iPR, not iCR) at the first timepoint, the original iCR should be converted to an iPR and the iBOR should be assessed as iPR.
- [c] Transient worsening of disease early in therapy or during temporary interruption of study therapy (eg, for drug-related toxicity or intercurrent illness) may not indicate true PD if followed by an iSD, iPR, or iCR

6.2 Secondary Efficacy Endpoints

6.2.1 Progression Free Survival

PFS is defined as the interval from the start of study therapy until the first documentation of disease progression or death from any cause (whichever occurs first). Disease progression may be radiological progression or clinical progression. The start of study therapy is the first dose date of eFT508.

Using RECIST v1.1, PFS will be calculated as:

- PFS (days) = Date of PD (radiological progression), clinical progression, or death (whichever occurs first) – first dose date of eFT508 + 1

Using iRECIST, iPFS will be calculated as:

- iPFS (days) = Date of iUPD (radiological progression), clinical progression, or death (whichever occurs first) – first dose date of eFT508 + 1

Radiological progression will be based on the Investigator assessment using RECIST v1.1/iRECIST.

The date of clinical progression will be defined as the date of study discontinuation due to clinical progression in the absence of radiological progression.

To be counted as radiological disease progression using iRECIST, the iUPD assessment must be confirmed as iCPD at the next iRECIST assessment. If an iUPD response is assessed but the progression is not confirmed (e.g. a later assessment of iSD, iPR, or iCR is recorded) then the initial iUPD will not be counted as an event for determination of iPFS. To be as conservative as possible, in the the case that an assessment of iUPD is recorded and no subsequent evaluable iRECIST assessments are recorded then the iUPD event will be treated as confirmed in the calculation of iPFS.

6.2.1.1 Censoring Rules for Progression Free Survival

The following rules will be applied for both PFS and iPFS.

Subjects who have not had radiological progression (i.e. an assessment of PD using RECIST or a previous response of iUPD using iRECIST that is confirmed iCPD at the next iRECIST assessment), have not experienced clinical progression, or died at the time of analysis will be censored at the time of the last response assessment. However, if the subject experiences progression (either clinical or radiological) or dies after 2 or more missed visits, the subject will be censored at the time of the latest response assessment prior to the two missed visits. In addition, subjects that do not have documented disease progression but begin subsequent anticancer therapy will be censored on the date the subsequent anticancer therapy is started.

If the subject has no evaluable visits or does not have a baseline assessment, they will be censored at Day 1.

The PFS time will be derived based on scan/assessment dates and not visit dates to determine radiological progression. Assessments/scans contributing towards a particular visit may be performed on different dates. The following rules will be applied for this calculation:

- Date of radiological progression will be determined based on the earliest of the assessment/scan dates of the component that triggered the progression (PD using RECIST or iUPD using iRECIST that is confirmed iCPD at the next assessment).
- When censoring a subject for PFS, the subject will be censored at the latest of the assessment/scan dates contributing to a particular overall visit assessment.

Overall visit assessments will be determined for each assessment (scheduled or unscheduled) and will contribute to the derivation of PFS.

6.2.2 Duration of Stable Disease

Duration of Stable Disease (DSD) will be calculated for all subjects achieving a BOR of SD or better (iBOR of iSD or better using iRECIST). DSD is defined as the interval lfrom the

start of study therapy until the first documentation of disease progression or death from any cause (whichever occurs first). Disease progression may be radiological progression or clinical progression. The start of study therapy is the first dose date of eFT508.

Using RECIST v1.1, DSD will be calculated as:

- DSD (days) = Date of PD (radiological progression), clinical progression, or death (whichever occurs first) – first dose date of eFT508 + 1

Using iRECIST, iDSD will be calculated as:

- iDSD (days) = Date of iUPD (radiological progression), clinical progression, or death (whichever occurs first) – first dose date of eFT508 + 1

To be counted as radiological disease progression, the iUPD assessment must be confirmed as iCPD at the next iRECIST assessment. In the event that an iUPD response is assessed but the progression is not confirmed (e.g. a later assessment of iSD, iPR, or iCR is observed) then the initial iUPD will not be counted as an event for determination of iDSD. To be as conservative as possible, in the the case that an assessment of iUPD is recorded and no subsequent iRECIST assessments are recorded (or are considered NE) then the iUPD event will be treated as confirmed.

The same censoring rules for PFS will be applied.

6.3 Safety Endpoints

6.3.1 Adverse Events

Adverse Events (AEs) will be coded using the Medical Dictionary for Regulatory Activities MedDRA (v21.0 or later). The severity of all adverse events will be graded according to NCI CTCAE version 4.03. All AEs regardless of causality or seriousness will be recorded from the date of first dosing until the later of either 30 days after the last dose of study therapy or until resolution/stabilization of any ongoing drug-related AEs.

In Part 2, the crossover period for subjects randomized to monotherapy and crossing over to the combination therapy arm following disease progression is defined as the day of the first dose of avelumab until the end of the AE follow-up period. The monotherapy period for these subjects will be defined as the date of first eFT508 dose until the day prior to the first dose of avelumab.

6.3.2 Dose Limiting Toxicities

In Part 1, a Dose Limiting Toxicity (DLT) is defined as an AE meeting specific criteria and not primarily related to the underlying malignancy, a comorbid condition, or an intercurrent illness. The full list of criteria for determination of DLTs is given in Protocol Section 3.2.3.

The incidence of DLTs will be monitored throughout Cycle 1 in order to help determine the MTD in Part 2.

6.3.3 Total Exposure to eFT508

The total exposure to eFT508 will be calculated as:

- Total exposure to eFT508 (days) = last dose date of eFT508 – first dose date of eFT508 + 1 – cumulative days of dosing interruptions

The duration of each dose interruption is defined as:

- Duration of dose interruption = End date of interruption – Start date of interruption + 1

The cumulative days of dosing interruptions is defined as the sum of the duration of dose interruptions.

If start/stop dates of dosing are missing after querying, the duration of treatment interruption will be treated as 1 day.

6.4 Pharmacokinetics

The pharmacokinetics of eFT508 and avelumab will be assessed from blood samples taken at time indicated in the Schedule of Events. Additional details regarding the PK parameters for eFT508 and avelumab will be described in a standalone PK analysis plan.

7 STATISTICAL ANALYSES

This section describes the statistical analyses to be conducted in relation to the primary, secondary, and exploratory objectives of the study.

7.1 General Statistical Considerations

All analyses will be performed using SAS® Version 9.3 or higher.

Unless specified otherwise, the analyses of data collected will be descriptive and summaries will be presented separately for each treatment arm and study part. As applicable, summaries may be also be presented in total for select analyses. Statistical analyses will be purely descriptive and no formal hypothesis testing or comparative analyses between treatment arms will be performed. Confidence intervals (CIs) will be constructed at the 95% confidence level where appropriate.

For continuous variables, the number of observations (n), mean, standard deviation, median, minimum, and maximum will be provided as summary statistics. For categorical variables, the frequency and percentage in each category will be displayed.

All Part 1 summary tables will display two treatment groups:

- eFT508 100mg BID and avelumab
- eFT508 200mg BID and avelumab

Part 2 summary tables will in general display three treatment groups:

- eFT508 200mg BID
- eFT508 200mg BID and avelumab
- eFT508 200mg BID crossover to eFT508 200mg BID plus avelumab

In the above Part 2 summaries, eFT508 200mg BID crossover subjects will be presented in the eFT508 200mg BID column for all safety assessments and AEs occurring prior to the first avelumab dose date. Any safety assessments and AEs occurring on or after the first avelumab dose date will be summarized in a separate column for crossover subjects. For analysis of efficacy data in Part 2, the analysis will be based on the randomized treatment arm (e.g. crossover subjects will be summarized in the monotherapy group).

Comprehensive data listings of all data will be generated. By-subject data listings of all clinical trial data will include all enrolled subjects.

7.1.1 Baseline Definition

Unless stated otherwise, the baseline value is defined as the last non-missing measurement prior to the first dose of eFT508. If a subject received avelumab but not eFT508 then the baseline value is defined as the last non-missing measurement prior to the first dose of eFT508.

Study Day 1 will be considered as the date of the first dose of eFT508.

7.1.2 Covariate Adjustment

No adjustment for other covariates is planned.

7.1.3 Multicenter Studies

The center effect will not be considered for this study.

7.1.4 Multiple Comparisons

No multiple comparisons adjustment will be applied to the efficacy analysis.

7.1.5 Handling of Dropouts or Missing Data

Unrecorded data values will be recorded as missing. Only recorded (i.e. complete) data values will be used for statistical analyses. In general, invalid or missing values will not be imputed unless stated otherwise.

In cases of missing or incomplete dates (e.g. AEs and concomitant medications), the missing component(s) will be assumed as the most conservative value possible. For example, AEs with missing start dates, but with stop dates either overlapping into the treatment period or missing, will be counted as treatment-emergent, taking the worst-case approach. When partial dates are present in the data, both a partial start date and/or a partial stop date will be evaluated to determine whether it can be conclusively established that the AE started prior to the start of study drug or ended prior to the start of study drug. If the above cannot be conclusively established based on the partial and/or present dates, then the AE will be considered as treatment-emergent. Actual data values as they appear in the original eCRFs will be presented in the data listings.

To be conservative in the case of missing causality assessment for AEs after data querying, AEs will be assumed to be related to eFT508.

In Part 2, for subjects that cross over from eFT508 monotherapy to combination therapy, if the AE cannot be conclusively determined to have occurred during the cross over period it will be assumed to have occurred during the monotherapy dosing period.

7.1.6 Examination of Subgroups

There is no planned subgroup analysis.

7.1.7 Crossover Subjects

Subjects randomized to eFT508 monotherapy in Part 2 may be eligible to crossover to the combination therapy treatment arm after 8 weeks of treatment and after radiographic disease progression (PD/iUPD) or who are no longer clinically stable.

7.1.8 Interim Analysis

No formal interim analysis is planned for Part 1.

Part 2 utilizes a Simon 2-stage mini-max approach to evaluate antitumor response in the combination arm (Simon 1989). The number of responses in the combination arm will be evaluated using iRECIST after the first subjects have been treated. If no responses are seen, randomization and enrollment in Part 2 will cease.

In the combination arm, the Simon 2-stage mini-max design to test the null hypothesis that the ORR is $\leq 5\%$ against the one-sided alternative hypothesis that it is $\geq 20\%$ will be used.

Under this design, 13 subjects will be accrued in the first stage. If 0 of 13 subjects have an objective tumor response per iRECIST, then randomization to both treatment arms will cease and the study will stop. If one or more objective tumor responses per iRECIST are seen in Stage 1, 14 additional subjects would be accrued in Stage 2 for a total of 27 subjects

7.2 Study Population Data

7.2.1 Subject Disposition

Subject disposition will be summarized for the Enrolled Population in each study part.

The following subject disposition categories will be summarized for Part 1:

- Subjects who enrolled;
- Subjects who received assigned treatment;
- Subjects discontinued from study treatment;
- Subject follow-up status at last follow-up visit; and
- Subjects who terminated the study.

The following subject disposition categories will be summarized for Part 2:

- Subjects who were randomized;
- Subjects who received randomized treatment;
- Subjects who crossed over from eFT508 monotherapy arm to combination treatment;
- Subjects discontinued from study treatment;
- Subject follow-up status at last follow-up visit; and
- Subjects who terminated from the study.

For subjects who discontinued study treatment and subjects who terminated the study, a summary will be provided by reason for discontinuation or termination.

The total number of subjects in each defined analysis population will be tabulated for each study part.

Subject disposition data will be listed by subject and study part.

7.2.2 Protocol Deviations

Protocol deviations deemed reportable in the the clinical study report (CSR) will be listed by subject for Part 1 and Part 2.

7.2.3 Demographics and Baseline Characteristics

Descriptive summaries of demographic and baseline characteristics will be presented using the Safety Population.

Demographic characteristics include, but are not limited to: age, sex, childbearing potential, race, ethnicity, body weight, height, and body mass index (BMI).

Cancer history will be collected during the Screening Period and summarized for the Enrolled Population. Continuous variables including time from original colorectal cancer diagnosis (months) and time from current colorectal cancer will be summarized with descriptive statistics. Categorical variables including original stage, current stage (overall, tumor, node, and metastasis), grade, primary tumor location, and colorectal adenocarcinoma type will be summarized.

7.2.4 Medical History

Medical history will be collected during the Screening Period. The reported medical history terms will be coded using the MedDRA v21.0 and presented in a summary table by System Organ Class (SOC) and Preferred Term for the Safety Population. Subjects may have more than one medical history for each SOC and Preferred Term. Subjects will be counted once within each SOC and Preferred Term classification.

7.2.5 Prior and Concomitant Medications

All medications taken by subjects between signing the informed consent form and the End of Therapy Visit will be recorded. Only concomitant medications taken for the treatment of an AE will be recorded for up to 30 days after the last dose of study therapy.

All concomitant medications will be summarized by Anatomical Therapeutic Chemical (ATC) and preferred term by treatment group and overall for the Safety Population in each study part. Although a subject may have taken two or more medications, the subject is counted only once within an ATC classification. The same subject may contribute to two or more preferred terms in the same classification.

7.2.6 Prior Cancer Therapies, Surgeries, and Radiation

The number and percentage of patients with previous cancer therapy/medications will be summarized by line of treatment and best overall response to all prior treatments for the Safety Population. The reported cancer therapy/medication terms will be coded using WHO Drug Dictionary (WHO-DDE B2, Version March 2017 or later). Prior cancer therapies/medications will be summarized by Anatomical Therapeutic Chemical (ATC) and preferred term by treatment arm (if applicable) in each study part. A subject will be counted only once within an ATC classification but may contribute to two or more preferred terms in the same classification.

7.3 Efficacy Analyses

Efficacy analyses described in this section will be performed using the Safety Population in Part 2.

7.3.1 Primary Efficacy Analyses

The primary analysis will involve analysis of the ORR/iORR by treatment arm. The point estimate of the ORR and iORR along with the exact Clopper-Pearson 95% CI will be presented. Subjects who do not have sufficient baseline and on-study tumor assessments to characterize response (i.e. who have a BOR of NE) will be included in the calculation of the ORR/iORR.

The best objective response (BOR/iBOR) will also be tabulated by treatment arm to show the number and percentage of subjects in each response category.

In addition, for subjects that have confirmed disease progression (iCPD) using iRECIST the type of disease progression will be tabulated. The following categories for disease progression will be summarized:

- Further increase in sum of target lesions $\geq 5\text{mm}$
- Further increase in sum of new target lesions $\geq 5\text{mm}$
- Another new lesion
- Significant further increase in non-target lesions –unequivocal increase
- Clinical Progression (as determined by the Investigator)

7.3.2 Secondary Efficacy Analyses

7.3.2.1 Progression Free Survival

PFS/iPFS will be estimated using the Kaplan-Meier method for each treatment arm in Part 2.

The number and percentage of subjects experiencing a PFS/iPFS event and the type of event (clinical progression, radiological progression, or death) and subjects censored will be summarized. The Kaplan-Meier estimate of the median PFS/iPFS and Brookmeyer-Crowley 95% CI will be presented (Brookmeyer and Crowley 1982)

Kaplan-Meier plots of PFS/iPFS will be presented by treatment arm in Part 2.

The analysis of PFS/iPFS will be based on the Investigator overall assessment and using all assessments, including unscheduled assessments.

7.3.2.2 Duration of Stable Disease

DSD/iDSD will be estimated using the Kaplan-Meier method for each treatment arm in Part 2.

The number and percentage of subjects experiencing a DSD/iDSD event and the type of event (clinical progression, radiological progression, or death) and subjects censored will be summarized. The Kaplan-Meier estimate of the median DSD/iDSD and Brookmeyer-Crowley 95% CI will be presented (Brookmeyer and Crowley 1982).

Kaplan-Meier plots of DSD/iDSD will be presented by treatment arm in Part 2.

The analysis of DSD/iDSD will be based on the Investigator overall assessment and using all assessments, including unscheduled assessments.

7.3.3 Immunohistochemistry of Tumor-Infiltrating Lymphocytes

All immunohistochemistry data will be listed.

7.3.4 Molecular Profiling of Tumor-Infiltrating Lymphocytes

All molecular profiling data will be listed.

7.3.5 Biomarkers of Immune Activation

All immune activation data will be listed.

7.3.6 PD-L1

All PD-L1 assessments data be listed.

7.3.7 eIF4E Measurements

All EIF4E assessments data be listed.

7.3.8 Genetic Mutations

All other genetic mutation data will be listed.

7.4 Safety Analyses

The assessment of the incidence of adverse events during the course of the study will consist of the surveillance and recording of adverse events (AEs), including serious adverse events (SAEs). Safety will also be assessed through clinical laboratory evaluations, vital signs, ECGs, physical examinations, ECOG performance status, and prior and concomitant medication reporting.

No formal statistical analysis of the safety data will be conducted. Summaries will be presented separately for Part 1 and Part 2. In each study part, all summaries will be tabulated by treatment arm for the Safety Population.

7.4.1 Extent of Exposure

Descriptive statistics will be provided for the total exposure to eFT508. The number and percentage of subjects administered 1, 2, 3, 4, 5, and 6 or more treatment cycles of eFT508 will be summarized. A cycle is defined as 28 days. In addition, the number of subjects with a

dose modification, missed dose, and dose interruption will be summarized including the reason for dose modification, missed dose, and dose interruption.

7.4.2 Adverse Events

In each study part, summaries of adverse events will be presented by treatment arm using the Safety Population with the exception of the DLT summary. Summaries of drug-related adverse events will be presented separately for eFT508 and avelumab as applicable.

An overview of treatment-emergent adverse events will be provided which summarizes the subject incidence of the following for the Safety Population in each study part:

- Any TEAEs,
- Any SAEs
- DLTs (Part 1 only, utilizing the DLT Evaluable Population)
- Drug-related TEAEs (related to eFT508, related to Avelumab, related to both),
- CTCAE grade 3/4/5 TEAEs,
- Drug-related CTCAE grade 3/4/5 TEAEs,
- Deaths,
- Treatment-emergent SAEs (TESAEs),
- Drug-related TESAEs,
- AEs leading to dose reduction or interruption of eFT508
- AEs leading to discontinuation of eFT508

The number and percentage of subjects with TEAEs during the AE reporting period will be tabulated by the highest CTCAE Grade, System Organ Class (SOC), and Preferred Term (PT). DLTs (part 1 only for the DLT Evaluable Population) and TEAEs related to eFT508 will be summarized in the same manner.

TEAEs will be summarized by SOC and PT. SAEs, CTCAE Grade 3/4/5 TEAEs, drug-related CTCAE Grade 3/4/5 TEAEs, Deaths, TESAEs, drug-related TESAEs, AEs leading to dose reduction or interruption of eFT508, and AEs leading to discontinuation of eFT508 will be summarized in the same manner.

TEAEs occurring during the crossover period will be summarized in the crossover treatment column in Part 2. If it cannot be conclusively determined that the AE occurred during the crossover period from the available data the AE will be summarized in the monotherapy column. AEs for crossover subjects will not be double counted between the monotherapy and crossover columns (i.e. each AE will be assigned to only one period) and will be counted only once for purpose of the total column.

For all above summaries, subjects with multiple adverse events will be counted only once per SOC and preferred term.

Listings will also be provided of deaths, SAEs, and AEs leading to discontinuation of eFT508.

7.4.3 Clinical Laboratory Evaluations

Clinical laboratory evaluations will be summarized using descriptive statistics for selected laboratory parameters (creatinine clearance, clinical chemistry, hematology, thyroid, coagulation, and urinalysis) including absolute measurements and changes from baseline by scheduled time of evaluation. Changes from baseline by scheduled time of evaluation will include last visit on-study (End of Therapy Visit), maximum post-treatment value, and minimum post-treatment value. Both scheduled and unscheduled post-treatment visits will be considered for the summaries of the maximum and minimum post-treatment values.

Abnormal laboratory results (hematology, chemistry, and coagulation) will be graded according to NCI-CTCAE version 4.03 as applicable. A shift table, presenting the 2-way frequency tabulation for baseline and the worst post-treatment value according to the NCI-CTCAE grade, will be provided for selected clinical laboratory tests. For select parameters (e.g. glucose), separate shift tables indicating hyper- and hypo-directionality of change may be produced. For laboratory parameters with no CTCAE grading, shift tables (with categories of low, normal, high) from baseline to post-treatment maximum and from baseline to post-treatment minimum may be produced if warranted after data review. Both scheduled and unscheduled post-treatment visits will be considered in tabulation of the worst post-treatment value.

In each study part, clinical laboratory summaries will be presented by treatment arm.

All clinical laboratory evaluations will be listed by subject and abnormal values will be flagged.

7.4.4 Vital Signs

In each study part, descriptive statistics will be provided by treatment group and time point for vital signs measurements (systolic and diastolic blood pressure, pulse rate, respiratory rate, body temperature, and weight). Changes from baseline by scheduled evaluation, and maximum and minimum post-baseline values will be presented. Both scheduled and unscheduled post-treatment values will be considered for summaries of the minimum and maximum.

7.4.5 Electrocardiograms

Electrocardiogram parameters (PR, RR, QRS, QT, QTcF) will be summarized using descriptive statistics for actual values and for changes from baseline by scheduled time of evaluation, including the Final Visit and the maximum post-treatment value. Both scheduled

and unscheduled post-treatment values will be considered for summaries of the maximum post-treatment values.

The Fridericia-corrected QT interval (QTcF) will be calculated as $QTcF = QT/(RR)^{1/3}$ for the above summaries.

In addition, subjects experiencing QTcF elevation or change from baseline will be tabulated by visit for each post-baseline evaluation and at any time post-baseline for the following categories:

- QTcF > 450 ms
- QTcF > 480 ms
- QTcF > 500 ms
- Increase from baseline QTcF > 30 ms
- Increase from baseline QTcF > 60 ms

ECG readings will be judged for overall clinical significance and tabulated by treatment group. Shift tables from baseline to the worst post-baseline result will be presented by treatment arm for each study part. The following categories will be used: Normal, Abnormal not clinically significant, Abnormal clinically significant.

7.4.6 Performance Status

For subjects with ECOG performance evaluation, the performance status at each visit on study will be tabulated by treatment group.

7.4.7 Other Safety Analyses

All other safety assessments (e.g. physical examination) will be listed.

7.5 Pharmacokinetic Analysis

Additional details regarding the PK parameters and methods of PK analysis for eFT508 and avelumab will be described in a standalone PK analysis plan.

8 GENERAL INFORMATION

8.1 Statistical Software

The creation of analysis datasets and statistical analyses will be done using SAS® version 9.3 or higher. The Medpace standard operating procedures (Medpace documents GL-DS-02-S3 and GL-DS-03-S2) will be followed for the validation of all SAS programs and outputs.

8.2 Format of Tables, Listings, and Figures

The format of tables, listings, and figures will be described in a stand-alone programming specifications document and will be prepared before database lock for the study.

9 CHANGES FROM PROTOCOL-SPECIFIED ANALYSIS

Table 6: Summary of Changes from Protocol-Specified Analysis

Protocol Section	Change	Rationale
Section 10.2.2: Part 2	Simon 2-stage design not utilized. The study was fully enrolled in Part 2 with no interim analysis.	Rapid study enrollment caused the study to be fully enrolled with no interim analysis.
Section 7.3.7: Immunogenicity	Immunogenicity assessment	If analyzed in the future, it will be presented separately as a standalone document.
Section 3.2.2: Definition of DLT Evaluable	Language added to clarify that subjects will not be included in the DLT Evaluable population if <44 of 56 doses of eFT508 were received during Cycle 1 in Part 1 due to <i>bid</i> dosing schedule of eFT508.	The planned administration of eFT508 was <i>bid</i> for 28 days during Cycle 1, resulting in 56 planned eFT508 doses during this period.
Section 10.4.4.3: Laboratory Evaluations	Language added to clarify that shift tables will be presented for select laboratory parameters based on CTCAE grade (if applicable, including possible bi-directionality) or normal ranges (if applicable).	Additional detail was provided for the safety analysis.
Section 10.4.5.1: Definition of Efficacy Endpoints	Removed analysis of Disease Control Rate.	Analysis removed due to lack of subjects demonstrating efficacy.

Section 10.4.5.1: Definition of Efficacy Endpoints	Removed analysis of Duration of Response.	Analysis removed due to lack of subjects demonstrating efficacy.
Section 10.4.5.1: Definition of Efficacy Endpoints	Removed analysis of Overall Survival.	Analysis removed due to lack of subjects demonstrating efficacy.
Section 10.4.5.1: Definition of Efficacy Endpoints	The definition of Progression Free Survival (PFS/iPFS) and Duration of Stable Disease (DSD/iDSD) was clarified to include clinical progression.	Clinical progression added to capture discontinuation of study due to clinical progression in the calculation of PFS/iPFS and DSD/iDSD.
Section 10.4.5.2: Analysis of Efficacy Endpoints	Sum of tumor measurements and change from baseline analysis removed. Efficacy analysis will be conducted by randomized treatment arm. Crossover subjects in Part 2 will be analyzed together with the monotherapy arm.	Analysis removed due to lack of subjects demonstrating efficacy.
Section 10.3: Analysis Populations	The Efficacy Evaluable Population was not defined in SAP. Efficacy analyses in Part 2 will be based on the Safety Population where applicable	Analysis removed due to lack of subjects demonstrating efficacy.
Section 10.4.2: Study Drug Exposure and Compliance	The study drug compliance analysis was clarified. Removed number (%) of cycles completed (%), number (%) of cycles delayed, number of doses of study drug taken, cumulative number of days of dosing,	Analysis removed due to lack of subjects with long-term exposure to study medication.

	cumulative dose in milligram analyses, number (%) of days study drug reported taken relative to the number of days study drug was expected to be taken over a specified period of time. The total exposure to eFT508, number of cycles initiated, and listings will be used to summarize study drug exposure and compliance.	
Section 10.4.7: Pharmacodynamic Analyses	Pharmacodynamic data will be presented in listings only. Table summaries were removed.	Analysis removed due to lack of subjects demonstrating efficacy.

10 REFERENCES

Brookmeyer, R., & Crowley, J. (1982). A confidence interval for the median survival time. *Biometrics*, 29-41.

Simon, R. (1989). Optimal two-stage designs for phase II clinical trials. *Contemporary Clinical Trials*, 10(1), 1-10.